

Dear Editor,

We submit for consideration at *The Lancet Global Health* our manuscript (Article) entitled “**Domestic Government Health Expenditure, Out-of-Pocket Spending, and Healthy Life Expectancy in 190 Countries, 2000–2022.**”

What we found. In a panel of 190 countries through 2022 (complete-case analyses on 189), we separate between-country development gradients from within-country changes in domestic government health expenditure (GHE). the pooled within-country association was null (TWFE $\beta = -0.12$, $p = 0.077$; one-sided exclusion testing rules out effects above $+0.10$ HALE-years per percentage point of GDP); the low-income subgroup showed a positive but statistically uncertain signal (wild cluster bootstrap $p = 0.065$); and GHE was consistently associated with lower out-of-pocket shares ($\approx 11\%$ relative reduction per percentage point of GDP, $p < 0.001$).

Relevance to *The Lancet Global Health*. This Article addresses a central policy question for universal health coverage: whether increases in domestic public health financing are associated with measurable gains in healthy life expectancy and financial protection, particularly in low-income settings. Our estimates suggest that the GHE-to-GDP ratio alone is not a strong predictor of short-to-medium-term HALE change overall, while public financing is consistently associated with lower out-of-pocket expenditure shares. The positive but uncertain low-income signal is, in our view, hypothesis-generating and directly relevant to forthcoming domestic financing commitments. Recent UHC monitoring documents slowing progress and persistent service gaps in low-income countries alongside continuing financial-protection challenges from out-of-pocket spending, while near-term ODA projections indicate further cuts by major donors, making evidence on the returns to domestic public financing more urgent.

Methods and transparency. Analyses were implemented through parallel pipelines with consistent conclusions, reporting follows STROBE and GATHER, and event studies, synthetic control case studies, and pathway decompositions are reported as exploratory. A complete anonymised replication package is available to editors and reviewers on request and will be deposited in Zenodo with a DOI. AI-assisted tools were used for language editing and structural suggestions; all output was reviewed and edited by the authors, who take full responsibility for the content, and no AI tool was used for data generation or statistical analysis.

190-country panel; 4,304 complete-case country-years (189 countries; 2000–2022); ~ 3335 words (main text); 3 tables, 3 figures; 20 supplementary tables, 14 supplementary figures. This study used publicly available, de-identified aggregate country-level data and did not require institutional review board approval.

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We confirm that this manuscript has not been published elsewhere and is not under consideration by another journal. All authors have approved the manuscript and agree with its submission to *The Lancet Global Health*.

Sincerely,

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